



Locking Open Ward Doors (Mental Health) Guideline

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

Guiding principles

- The purpose of this policy is to ensure a safe and appropriate environment is maintained for staff, patients and visitors. Policy conforms to the intent of the Western Australian [Mental Health Act 2014](#) and [Charter of Mental Health Care Principles](#).
- To ensure a safe environment for staff, patients and visitors, the entry doors of the Adult Open ward at Mimidi Park are locked between 20:00 - 08:00hrs daily.
- In exceptional circumstances it may be necessary to lock entry and access doors to the Adult Open ward. This action should only be undertaken when all other less restrictive alternatives have been exhausted.
- The Adult Closed ward can be utilised temporarily to manage clinical risk or acuity for open ward patients.
- Once the Open ward is locked, staff must consider all strategies to re-open the ward as soon as possible.

Locking Open Ward doors

- Locking the Adult Open ward includes the locking of all doors inclusive of the entrances to the ward and all courtyards.
- During normal business hours, the locking of the Adult Open doors can be authorised by the Nurse Unit Manager (NUM), Clinical Nurse Specialist (CNS) or the Shift Coordinator in consultation with the treating medical officer (MO) or treating Consultant.
- The Shift Coordinator can lock the adult open doors but must immediately notify the Nurse Unit Manager (NUM), Clinical Nurse Specialist (CNS).

RESPONSIBILITY				NATIONAL STANDARDS National Standards for Mental Health 1, 2, 6, 10
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- Outside of normal business hours, the closure of the Adult Open ward can be authorised by the Shift Coordinator in consultation with the Duty Medical Officer (DMO). The Shift Coordinator will notify the Duty Nurse Manager (DNM) and the DMO will notify the On-Call Consultant.
- This action should only be undertaken following consideration of less restrictive options such as increasing visual observations, placing a patient on a special or temporarily transferring the patient to the closed ward.
- If the patient is voluntary and wants to leave the authorised hospital, the Shift Coordinator should facilitate an urgent medical review. If there is an immediate risk, the Shift Coordinator should consider use of the Mental Health Act 2014 and utilisation of Form 2 'Order to detain voluntary inpatient in an authorised hospital for assessment'
- The Shift Coordinator is to email '**PaRK MHS ALL Staff**' and '**RPG RGH Duty Nurse Managers**' to inform them when the doors are locked; and when the doors reopen; and/or the outcome of any review of the locked doors.
- When the Adult Open ward doors have been locked this is to be documented in the 24 hour report by the nursing night Shift Coordinator.
- All consumers will be advised that the doors are required to be locked for clinical reasons and that accessibility should be via their allocated nurse. Signage should be placed on the external and internal of all affected doors, advising that the doors are to be secure on exit and entry. The locking of the doors does not impede the access to and from the ward for voluntary patients as per their care plan. Refer to [SMHS Granting and management of leave for mental health inpatients](#).
- A patient who is not allowed to leave the ward must have access to support and advice during this time.
- An immediate review of the patient's management plan and a new risk assessment should then be undertaken with the treating Consultant Psychiatrist or delegated medical registrar to determine if further action under the Mental Health Act 2014 is required.
- If Adult Open ward doors are locked up until 20:00hrs then the 'locked doors' are considered ceased.
- The clinical team should discuss at morning handover continuation of locking the doors and potential mitigating strategies to manage behaviours of concern.
- If the doors remain locked, notification, is to be made by the Shift Coordinator via email to '**PaRK MHS ALL Staff**' and '**RPG RGH Duty Nurse Managers**'
- It is essential to ensure that the notification process regarding the times the doors are locked outside of the normal security locked doors process continues, especially if this period coincides with public holidays or weekends. This is to ensure there is no misinterpretation of the length of time the doors are locked or the rationale for why they are locked.
- In the event there is a breach of security, the Shift Coordinator will complete a head count of patients. If the patient at risk is noted to be missing, the Shift Coordinator will implement the [WA Department of Health Missing Person Policy](#) in conjunction with the [SMHS Service Missing or Suspected Missing Mental Health Patients Policy](#)

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([SMHS:47](#)) and the [RKPG Missing or Suspected Missing Patients Policy and Procedure](#).

- If the Adult Open ward entry door has been locked to prevent entry of a specific person, and this person will not leave the premises, security must be called to escort this person off the premises. Refer to [RKPG Visitor Behaviour Management at Rockingham Peel Group Premises Policy and Procedure](#).
- Staff should also refer to the; [Flowchart of response to Missing or Suspected Missing Mental Health Inpatient](#)

Transfer of high-risk absconding patient to closed ward

- At times it may be an option to transfer a patient for a short period of time to the closed ward for further assessment and mitigation of immediate risks.
- A decision can be made by the Shift Coordinator as to whether transfer to the closed ward is a suitable **option**.
- Consideration of closed ward acuity and associated risk should be included in the rationale to transfer.
- Ideally a plan for safe transfer should be formulated, the plan should consider; gender, trauma and cultural considerations and whether **Security should be asked to attend** and assist in transfer. If the patient is voluntary, the nurse in charge must complete Form 2 'Order to detain voluntary inpatient in authorised hospital for assessment' (Mental Health Act 2014).

The following positions must be informed within business hours (08:00 - 16:00hrs):

- * The NUM.
- * The CNS.
- * The treating Consultant or Medical delegate.
- The following positions must be informed outside of business hours (16:00 08:00hrs):
 - * The DMO.
 - * The DNM.
- The patient must be reviewed within two hours by the treating team or on-call DMO after hours.
- The review will include revision of the patient's management plan, risk assessment and Mental Health Act status
- A contemporaneous entry must be made in the Digital Medical Record highlighting the reasons and risk identified which justify the transfer to the closed ward, all persons informed of transfer and the ongoing plan for treatment
- Consideration should be given to contacting the patients next of kin or carer.

Compliance monitoring

Compliance will be monitored by any clinical incidents via Datix

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References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. (Version 2) 2021. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>

Links

[Department of Health \(DoH\) Missing Person Policy \(MP0012/16\)](#)

[Government of Western Australia \(WA\) Mental Health Act 2014](#)

[National Standards for Mental Health Services](#)

[RKPG Missing or Suspected Missing Patients Policy and Procedure](#)

[RKPG Visitor Behaviour Management at Rockingham Peel Group Premises Policy and Procedure](#)

[SMHS Rights of Patients in Mental Health](#)

[SMHS Granting and Management of Leave for Mental Health Inpatients Policy](#)

[SMHS Missing or Suspected Missing Mental Health Patients](#)

[WA Charter of Mental Health Principles MHA 2014](#)

[WA Chief Psychiatrists Standards for Clinical Care](#)

National Standards for Mental Health Services

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